

Making sense of AI in the clinic

THE TALK

An invited talk for trauma and orthopaedic physiotherapists in the ATOCP session at the British Orthopaedic Association Annual Congress. It covers what AI can already do in clinical work, how clinicians tend to respond to it, and what they bring that AI systems don't have.

What AI can already do

In head-to-head studies, AI has diagnosed more accurately than doctors and written replies to patients that clinicians rated as more empathetic.

Sanctuary strategies

Denial, retreat, restriction, and resignation: four common responses that leave clinicians watching the change happen to them.

The context you bring

AI has no memory of you and no knowledge of your patients, so the context you give it is what makes its output useful.

Patients bring context too

Patients already use AI before they arrive, and their own agents will carry their history and goals into the consultation.

CONCEPTS

Context sovereignty

Controlling your own professional context (your specialism, your patients, local pathways, and how you reason) and using it to shape what AI does for you, whichever provider you use. Your patients hold the same claim over theirs.

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Distributed cognition

Clinical reasoning is shared across people and tools: the team, the records, the patient, and now AI. Your judgement shapes how well that whole system thinks, including what the AI sees and what gets checked.

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AI agent

An AI system that keeps context across time, plans a sequence of steps, and acts: it retrieves records, drafts notes, and books follow-ups. Your patients will increasingly bring their own into the consultation.

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KEY TAKEAWAYS

01

Write down your professional context

Your specialism, patient population, CPD priorities, and how you reason, on one page and explicit enough to give to an AI. Paste it in before you ask for anything clinical, and add to it as your practice changes. Keep patient-identifiable information out of it.

02

Give one patient a structured prompt

Ask a patient to put one question to their AI before the next appointment: what should I be asking about my progress and the next phase of treatment? It helps them use AI well and gives you both a better conversation to start from.

03

Treat AI output as a colleague's opinion

Engage with it, test it against what you know about the patient in front of you, and then decide. AI makes different errors from the ones people make, so the checking stays with you, and so does the synthesis.

04

Ask patients what their AI told them

Make it a routine question at the start of an appointment. It brings the patient's own research into the open, shows you what they already believe, and gives you the chance to correct it or build on it.

FURTHER READING

Context sovereignty for AI-supported learning: the longer argument for why the context you bring, and who controls it, decides what AI is worth in practice. It's written about education, and the argument carries over to the clinic.

Context engineering for educators: the practical side of giving AI the context it needs. A prompt is the instruction, and context is everything else the model works with.

We're comparing AI chatbot health advice to the wrong thing: one in seven people in the UK use AI chatbots for health advice instead of seeing a GP. Why warning patients off is the wrong response, and what a better one looks like.

How to replace a physiotherapist: Rowe, Nicholls, and Shaw (2022) on how AI redistributes the work of physiotherapy, and which parts of practice are most exposed.